

Riverside General Hospital

123 Wellness Ave, Springfield | (555) 019-2837 | This is a fictional sample document for testing purposes only

Discharge Summary

Patient Name:	Jordan A. Whitfield	Patient ID:	RGH-2026-004821
Date of Birth:	March 14, 1985	Sex:	Male
Admission Date:	June 18, 2026	Discharge Date:	June 23, 2026
Attending Physician:	Dr. Elena Marsh, MD	Department:	Internal Medicine

Reason for Admission

Patient presented to the Emergency Department with a three-day history of persistent epigastric pain, low-grade fever (38.2°C), and reduced appetite. Pain was described as dull and constant, worsening after meals, with no radiation. No prior history of similar episodes.

History of Present Illness

Mr. Whitfield is a 41-year-old male with a past medical history of Type 2 Diabetes Mellitus (diagnosed 2019, managed with Metformin) and mild hypertension. He reported no recent travel, no significant alcohol use, and denied any NSAID overuse. Initial labs showed mildly elevated white blood cell count and elevated lipase, prompting further imaging.

Diagnostic Findings

Test	Result	Reference Range
White Blood Cell Count	12.4 x10 ⁹ /L	4.0 - 11.0 x10 ⁹ /L
Serum Lipase	310 U/L	10 - 140 U/L
Fasting Blood Glucose	168 mg/dL	70 - 100 mg/dL
HbA1c	7.8%	< 5.7%
Abdominal Ultrasound	Mild pancreatic edema, no gallstones seen -	

Diagnosis

- Acute mild pancreatitis, likely metabolic in origin
- Type 2 Diabetes Mellitus, suboptimally controlled
- Essential hypertension, stable

Hospital Course

Patient was admitted for bowel rest, IV fluid resuscitation, and pain management. Blood glucose was closely monitored and controlled with adjusted insulin coverage during admission. Symptoms improved steadily over 72 hours, and the patient tolerated a gradual return to oral intake without recurrence of pain. Repeat lipase trended

down to 95 U/L prior to discharge.

Discharge Medications

Medication	Dosage	Frequency
Metformin	1000 mg	Twice daily with meals
Insulin Glargine	12 units	Once daily at bedtime
Pantoprazole	40 mg	Once daily before breakfast
Amlodipine	5 mg	Once daily

Follow-Up Instructions

Patient advised to follow up with Endocrinology in 2 weeks for diabetes management review, and with primary care physician in 1 week. Recommended low-fat diet during recovery period and gradual reintroduction of normal diet as tolerated. Patient counseled on warning signs requiring immediate return to ER: worsening abdominal pain, persistent vomiting, or fever above 38.5°C.

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